Social Media: Do's and Don'ts

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Importance

Today, use of digital media has become ubiquitous for education, social contacts, and entertainment. Adolescents are the prime users of digital media including social media. Children and adolescents who use social media may experience both positive and negative psychological outcomes.

- ☑ Social connectedness is important for psychosocial development in terms of sense of belonging, psychosocial wellbeing, and identity development and processes. Social media plays an important role in social connectedness. Social media also helps in enhancing knowledge and expression, exchange of resources, and information.
- ☑ Social media may also be harmful as it leads to problematic media/Internet use, reduced time for other activities, and potential for cyberbullying and other harms. Studies have demonstrated that low use (<1 hour per day) of social media is more beneficial than no use. However, moderate-to-heavy use (>2 hours per day) is harmful in various ways including for suicidal thoughts and plans.

Social media sites are extremely popular. While most adolescents use WhatsApp and Facebook, many are also on Twitter, Instagram, Snapchat, TikTok, and others. We, the pediatricians, should be able to help and guide the adolescents and their parents in safe use of social media and other digital media to aid adolescent health and development.

Benefits of Social Media

- ☑ It helps children to keep in touch with distant family members and friends.
- ☑ Students can work together on projects.
- ☑ It helps adolescents with conditions such as obesity, chronic illnesses such as diabetes mellitus or depression, chronic disabilities, or rare diseases, to network with each other.
- ☑ It fosters social inclusion among adolescents who feel excluded, such as those identifying as lesbian, gay, bisexual, transgender, queer, and intersex (LGBTQI).
- ☑ It aids in the spread of information, innovations, and ideas.
- ☑ It creates opportunities for community activism.

Risks Associated with Social Media

Standard Treatment Guidelines

- ☑ It decreases physical activity and contributes to the development of overweight and obesity.
- ☑ Sleep problems, including disturbed sleep and sleep deficit due to delay in going to bed or difficulty in falling sleep.
- ☑ Social media addiction, resulting in inadequate time for sleep, study, physical activity, and real-time socialization.
- ☑ Exposure to ads promoting the use of alcohol, tobacco, and drugs.
- ☑ Exposure to sites promoting sexual license and disordered eating habits.
- ☑ Low feeling brought on by persistent viewing of online posts, which give the impression that everyone else than you are always successful and enjoying pleasurable experiences.
- ☑ Risk of cybercrime, including cyberbullying, sexting, and online grooming for child abuse.
- ☑ **Loss of privacy:** Information once shared on a post may spread easily to unintended recipients and may be impossible to remove.

Standard Treatment Guidelines

The Pediatrician's role is primarily that of a guide to parents and a counsellor to children. He should be aware of the tools used to screen for sexting, cyberbullying, and problematic Internet use. His advice to parents are as follows:

☑ **Limit screen use:**

- Screen time for entertainment should be restricted. Parents should negotiate with adolescents to keep it minimum. Help the parents construct a Family Media Use Plan, making a daily 24-hour timetable for each adolescent that includes school time and homework, 8–12 hours for sleep, 1 hour for physical activity, and time for family, friends, and relaxation.
- No use of media while studying or at meals.

☑ **No secretive online activity:**

- The computer should be used in a public area; no devices in the bedroom.
- No device use from an hour before bedtime.
- Discuss and decide the utility and restricted use of smartphones for school-going children with monitoring the use of it.

☑ **Monitor their online activity:**

- *Before 10 years:*
  - Do not permit the use of social media at all before 10 years, except for video chatting with relatives.
- **Early-to-middle adolescence:**
  - Discourage the use of social media except for educational, sports, and extracurricular assignments. Check their online history regularly.
  - Talk to them daily about the sites they visited, whether they chatted with anyone, whether they made any post on social media.
  - Monitor their email for offensive content.
  - Discuss your own online practices with them.
- **Late adolescence:**
  - They demand, and need, more privacy.
  - Try to ask them daily about their social media use, but do not expect the whole truth.
  - Do ask if he/she felt uncomfortable with a message or post by any known or unknown person.

Standard Treatment Guidelines

☑ **Visit them online on social media sites:**

- Familiarize yourself with the social media site and confirm that it is appropriate for them. Discuss these with them and warn them of potential harms.
- Join them on the site and compulsorily "friend" them.
- Teach them how to use the privacy features of the site.

☑ **Teach them the safety rules:**

- *Beware of strangers:*
  - Use social media sites only for strengthening friendships, not for making new virtual friends.
  - Chat online only with those you know personally.
  - Never exchange photos with strangers.
  - Never give out personal information to a stranger, not even name, age, address, phone number, date of birth, personal interests, name of the school, or friends' names.
  - Do not meet in person someone whom you only know online. If you wish to meet an online friend, meet the person along with a parent (or at least a friend, in a public place).
- *Never respond to uncomfortable or hurtful messages.* Immediately stop the communication, shut down the computer, and inform your parents.
- *Never reveal passwords*, not even to a boyfriend, girlfriend, or best friend. This is important to prevent theft of money and identity theft. Contact 1930 for Emergency Financial Cyber Crime or 112 for all other cybercrime emergencies.

☑ **Discuss cyberbullying:** Never share something online that can hurt someone, not even as a joke, because lots of people will read it, and they may pass it on to more people. Do not gossip, spread rumors, or damage someone's reputation online.

☑ **Discuss sexting:**

- A girl may sext as a way of flirting, a joke, to become popular, or to be seen as cool. The delighted and excited boy may share his girlfriend's photo with his two best male friends—and one of them may post it online.
- A boy may take revenge on his ex-girlfriend by posting an intimate image, video, or text. Remember that 99% of adolescent love affairs do not last.
- Virtual identities may be fake. Even the pictures may be of someone else.

Risks Associated with Social Media

☑ **Use parental controls and filtering software:**

- Basic digital controls on all smartphones tell you how much time was spent every day on each App site.
- Google Family App is free, and informs the parent of what Apps are loaded on his child's phone, and every new App he uploads, and can block any App, or restrict the use of an App to a given time.
- Paid Apps such as Net Nanny automatically block all pornographic, violent, dangerous, and filthy sites, and give the parent dictatorial powers over the child's online usage.

Further Reading

☑ Council on Communications and Media. Media Use in School-aged Children and Adolescents. Pediatrics. 2016;138(5):e20162592.

☑ Gupta P, Shah D, Bedi N, Galagali P, Dalwai S, Agrawal S, et al. Indian Academy of Pediatrics Guidelines on Screen Time and Digital Wellness in Infants, Children and Adolescents. Indian Pediatr. 2022; 59:235-44.

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Approach to Short Stature

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Approach to Short Stature

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Definition

- ☑ Height 2 standard deviation (SD) or more below the mean height for that gender and chronological age (CA) in a given population.
- ☑ Height >1.5 SD below the midparental height (MPH).
- ☑ Fall in growth velocity below the 25th percentile.

Etiological Classification

Physiological	***Pathological***
Constitutional delay in growth and puberty (CDGP)	*Endocrine* ☑ Hypothyroidism ☑ Growth hormone (GH) ☑ Cushing ☑ Pseudohypoparathyroidism ☑ Precocious puberty ☑ Insulin-dependent diabetes mellitus (IDDM)—poorly controlled ☑ Multiple pituitary hormone deficiency (MPHD) maybe genetic/acquired
Genetic/ familial	*Nutritional* ☑ Hypocaloric/hypoproteinemic ☑ Rickets
	Small for gestational age (SGA) with failure to catch up
	Skeletal dysplasias and chondrodystrophies
	Chromosomal defects/syndromic—Turner, Down's, Noonan, Prader–Willi, and Russell–Silver
	Iatrogenic—intracranial trauma, tumor, irradiation, and surgery Medications—glucocorticoids and chemotherapy
	Idiopathic short stature

Evaluation

History

- ☑ Newborn period—low birth weight (SGA), hypoglycemia/jaundice [GH deficiency (GHD)], feeding difficulties (Prader–Willi/Russel–Silver), and swelling extremities (Turner)
- ☑ Symptoms pertaining to chronic illness—chronic diarrhea, greasy stools (malabsorption), polyuria, polydipsia [renal tubular acidosis (RTA)], and chronic respiratory or cardiac symptoms
- ☑ History of psychosocial deprivation
- ☑ Nutritional history
- ☑ History of lethargy, constipation, weight gain (hypothyroidism)
- ☑ History of headache, vomiting, and visual disturbances (MPHD)
- ☑ Family history of short stature, precocious puberty, delayed puberty [constitutional delay of growth and puberty (CDGP)], and consanguinity

Clinical Pointers

Skull	☑ Anterior fontanelle (AF)—wide in rickets, hypothyroidism, achondroplasia, osteogenesis imperfecta, mucoploysaccharidosis, etc. ☑ Microcephaly—syndromic short stature and intrauterine infections
Facies	☑ Frontal bossing/midfacial hypoplasia/immature facies/depressed nasal bridge, midline defects, and micropenis—GHD; facial puffiness/coarse facies/goiter—hypothyroidism ☑ Low posterior neck line/neck webbing/cubitus valgus/pigmented nevi—Turner; almond-shaped eyes/obesity; acromicria—Prader–Willi; Moon facies—Cushing; triangular facies—Russell–Silver
Eyes	☑ Cataract, corneal clouding, visual defects, optic atrophy, MPHD, and septo-optic dysplasia
Teeth	☑ Delayed dentition—rickets, hypothyroidism, and GHD ☑ Defective enamel—mucopolysaccharidosis (MPS), rickets, and hypothyroidism ☑ Single central incisor—multiple pituitary hormone deficiency
Neck	☑ Turner—webbed neck and hypothyroid—goiter
Skin	☑ Dry skin/myxedema—hypothyroidism and striae—Cushing
Limbs	☑ Deformities—skeletal dysplasia, rickets, and MPS ☑ Clinodactyly—Russel–Silver ☑ Small hands and feet—GHD and Prader–Willi ☑ Short 4th metacarpal or metatarsal—Turner/pseudohypoparathyroidism/ Short stature homeobox-containing gene (SHOX) ☑ Delayed tendon reflexes—hypothyroidism
Genitalia	☑ Sexual maturity scoring and cryptorchidism ☑ Micropenis—GHD
Musculature	Asthenic in chronic illness and malnutrition
Hypertension	Cushing/chronic renal disease